



Interfacility Ambulance Transfer Request Form

PATIENT IMPRINT

- Instructions:**
1. This form must be completed by the physician, it will help to identify what type of transport is required
 2. Fax this completed form to the ambulance service provider and provide a hard copy to the transport team.
 3. **ALS/ALS-RN transfers must be conducted by Medic Ambulance (Solano County EOA Provider) 707-644-8989**

Patient Diagnosis: _____ Patient Allergies: _____

Medicare/Medi-Cal Physician Certification Statement

The undersigned practitioner certifies that they have personal knowledge of the patient's condition at the time transport is ordered and is medically necessary as specified above. This is not a guarantee of coverage or payment. (Form may be signed by MD, DO, RN, CM, NP, NS, PA if Medicare. If **Medi-Cal**, form must be signed by Physician, i.e. MD, DO.)

Signature _____

Date _____

Printed Name & Credentials _____

NPI Number _____

Patient Condition:

☐ Critical

☐ Noncritical

☐ BLS

- ☐ Supplemental oxygen Delivery type _____ Rate _____
- Medical reason for O2 _____
- Reason unable to self-maintain O2 _____
- ☐ Isotonic IV solution @ TKO rate (NOT ON PUMP)
- ☐ 5150 psychiatric hold
- ☐ Restraints
- ☐ Dementia requiring behavioral monitoring
- ☐ Isolation precautions
- ☐ Aspiration precautions
- ☐ Sedated, including narcotics within last 30 minutes
- ☐ Post-surgical positioning or movement precautions (fractures, decubitus ulcers, etc.)
- ☐ Bariatric patient: Weight: _____ Height: _____
- ☐ Other devices that require medical monitoring: _____
- Explain _____

☐ ALS (MUST GO TO EOA PROVIDER)

- ☐ Paramedic level assessment & decision making
- ☐ IV solution <40 mEq/L of Potassium Chloride (KCL)
- ☐ Cardiac monitoring
- ☐ Standby external cardiac pacing
- ☐ Continuous positive airway pressure (CPAP)
- ☐ Nebulizer therapy
- ☐ One or more ALS medications: adenosine, aspirin, atropine, beta-2 agonist bronchodilators, calcium chloride, dextrose, diphenhydramine, epinephrine, fentanyl, glucagon, lidocaine, midazolam, morphine, naloxone, nitroglycerin tablets/spray, sodium bicarbonate
- ☐ Blood product infusion
- ☐ Pump infusion of amiodarone, NTG, magnesium, or heparin
- ☐ Pump infusion of any isotonic IV solution (NS, LR, D5W, etc.)

☐ ALS-RN (MUST GO TO EOA PROVIDER)

- ☐ Nursing level assessment or decision making
- ☐ Medication(s) other than ALS medications listed above
- ☐ Medication(s) on an infusion pump not listed in ALS section
- ☐ Blood product infusion

☐ CCT

- ☐ **Critically ill or injured** – requires physician's initials: _____
- ☐ Ventilator management
- ☐ Invasive pressure monitoring devices (ex. CVP, Swan-Ganz, arterial line, ICP monitor, etc.)
- ☐ Transvenous pacing
- ☐ Intra-aortic balloon pump
- ☐ Extra corporeal membrane oxygenation
- ☐ High-risk L&D that may lead to neonatal critical care
- ☐ Neuromuscular blocking agents
- ☐ Continuous infusion of sedative agents (ex. propofol)

Requested response level:

☐ STAT

☐ Immediate (60 min)

☐ Scheduled (1-4 hrs)

☐ Planned (4-72 hrs)

Additional Doctor's Orders:

☐ PALS/ACLS/NRP protocols

☐ If patient needs services not available at sending facility, please specify: _____

☐ See attached order sheet for additional orders ☐ Other orders _____

EMS Time of Request _____

EMS Time of Arrival _____

Receiving Facility _____

Receiving Physician _____

Date and Time _____

Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Patrick O. Duterte, Director

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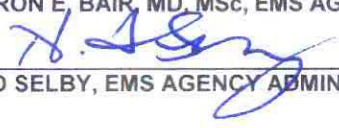
POLICY MEMORANDUM 7200

Implementation Date: June 5, 2013

Release Date: June 5, 2013

REVIEWED/APPROVED BY:


AARON E. BAIR, MD, MSc, EMS AGENCY MEDICAL DIRECTOR


TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: INTERFACILITY TRANSFER GUIDELINES

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5; §1797.220.

PURPOSE/POLICY: To establish guidelines for interfacility patient transfers.

I. DEFINITIONS:

- A. Emergency Medical Technician (EMT): Emergency Medical Technician means a person who has successfully completed a basic EMT course and who has been certified. EMT's may be used to transport Basic Life Support (BLS) level patients based on their scope of practice.
- B. Paramedic: Paramedic means a person who is educated and trained in all elements of Pre-hospital Advanced Life Support (ALS) and who has been licensed by the state. Paramedics may be used to transport patients based on their scope of practice.

- C. Registered Nurse: A Registered Nurse (RN) is a nurse who has graduated from a Nursing program at a college or university and has passed a National licensing exam. RNs may be used to transport patients based on their scope of practice.
- D. Critically Ill: Patients suffering from a life-threatening condition in which they are either critically ill or injured.
- E. Scope of Practice: As defined in California State Regulations and Solano County EMS Policies and Protocols:
 - 1. EMT: Title 22, Chapter 2, Article 2, §100063, and Policy 6300.
 - 2. Paramedic: Title 22, Chapter 4, Article 2, §100146, and Policy 6400.
 - 3. RN: Business and Professions Code §2725.

II. BASIC LIFE SUPPORT (BLS) TRANSPORT

- A. EMTs may transport patients performing functions or procedures falling within their Scope of Practice as outlined in Policy 6300.
- B. Hospital Staff should consider the EMT Scope of Practice when making patient transport decisions.
- C. EMTs must have a completed Inter-facility Transport Form (IFT) prior to transporting a patient.
- D. Request for EMT transport beyond scope of practice:
 - 1. In the event an EMT ambulance is dispatched to an acute care facility (i.e., a hospital) to perform an interfacility transfer and upon arrival the EMT determines that a RN or MD is not accompanying the patient, it is the responsibility of the EMT attending to the patient to inform the transferring physician (or his/her designee) that the patient's medical needs are beyond the EMT Scope of Practice. The transferring physician shall arrange for appropriate personnel to accompany the patient.
 - 2. In the event an EMT ambulance is dispatched to a non-acute care health facility (e.g., convalescent home, doctor's office) to transport a patient and the EMT determines that the patient may require a higher level of care, the EMT shall make a clinical decision to either immediately "load and go" and transport the patient to the closest appropriate hospital or to dispatch an Advanced Life Support (ALS) unit to the scene. This decision shall be based on the EMTs assessment of what immediate medical care is in the patient's best interest.
 - 3. Once the patient is turned over to the care of a physician at a Receiving Hospital, the EMT shall complete a Patient Care Report (PCR) and a Field Advisory Report (FAR) and notify the EMS Office via telephone or FAX of the occurrence. A copy of the PCR shall be submitted with the FAR.

III. ADVANCED LIFE SUPPORT (ALS) TRANSPORT

- A. Paramedics may transport patients performing functions or procedures falling within their scope of practice as defined in Policy 6400.
- B. Hospital Staff should consider Paramedic Scope of Practice when making patient transport decisions.
- C. In certain instances, RNs may be used to transport ALS patients. This level of transport is for patients whose condition exceeds the paramedic scope of practice, but the patient is not critically ill or injured as determined by the transferring physician.
 - Patients that may require this level of transport include but are not limited to intravenous (IV) antibiotics and other IV medicines outside of a paramedic's scope of practice.
- D. Paramedic and RN transports must have a completed IFT form prior to transporting a patient.

IV. CRITICAL CARE TRANSPORT (CCT)

- A. Critical Care Transport (CCT) teams perform interfacility transport moving critically ill or injured patients from one hospital to another for potentially advanced care. Patients include but are not limited to those requiring ventilators, intra-aortic balloon pumps, etc.
- B. CCT teams must have a completed IFT form prior to transporting a patient.

V. INTERFACILITY TRANSFER FORMS (IFT)

- A. Properly completed IFT forms will include the patients' name, level of care requested, EMS request time, EMS arrival time, diagnosis, receiving facility, receiving physician name, and discharge physician or staff nurse name and signature.
- B. It is the responsibility of the sending facility to provide the transporting ambulance staff an IFT form properly completed and signed by the transferring physician or staff nurse.
- C. It is the responsibility of the transferring ambulance staff to obtain an IFT form and to ensure the form is properly completed and signed by the transferring physician or staff nurse; and not to transport the patient until the form is properly completed and signed.

VI. SPECIAL PROCEDURES AND/OR EQUIPMENT

- A. For ambulance transportation from a residence, the patients will be transported to the nearest appropriate facility with intervention prescribed by EMT/Paramedic Scope of Practice and Solano County Policies and Procedures.

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POLICY MEMORANDUM 7210

Implementation Date: May 21, 2023
Review Date: March 1, 2025

REVIEWED/APPROVED BY:

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BENJAMIN GAMMON, EMT-P, EMS AGENCY ADMINISTRATOR

**SUBJECT: ADVANCED LIFE SUPPORT (ALS) PARAMEDIC INTERFACILITY
TRANSFER (IFT) OPTIONAL SKILLS TRANSFERRING HOSPITAL
REQUIREMENTS**

AUTHORITY: Health and Safety Code, Division 2.5, Sections 1798.200, 1798.206,
1798.214, 1797.218, 1797.220, 1798.2, 1798.170, and 1798.172
California Code of Regulations, Title 22, Chapter 4, Article 1, Section
100146

I. PURPOSE:

- A. To establish transferring hospital requirements for the utilization of any of the following Paramedic IFT optional skills:
1. Monitoring of magnesium sulfate, nitroglycerin (NTG), heparin, and/or amiodarone infusions.
 2. Monitoring of blood transfusions.
 3. Utilization of automatic transport ventilators (ATV).

II. PARAMEDIC IFT OPTIONAL SKILLS AND PATIENT REQUIREMENTS

- A. Only the Solano County ALS Exclusive Operating Area (EOA) Provider may be authorized to utilize Paramedic IFT optional skills.
- B. Only appropriately trained Paramedics employed by the ALS EOA Provider may utilize Paramedic IFT optional skills.
- C. The following criteria apply to patients that are candidates for the utilization of Paramedic IFT optional skills:
 - 1. Magnesium sulfate, nitroglycerin (NTG), heparin, and/or amiodarone infusions.
 - a. The infusion will have been running in a peripheral or central IV line for at least 10 minutes prior to transport.
 - b. Patients will have maintained stable vital signs for the previous 30 minutes AND will not have more than two medication infusion lines running exclusive of potassium chloride concentrations authorized under the paramedic basic scope of practice.
 - c. Timeframes indicated above will not apply to patients who require immediate transport for critical interventions when the transferring physician determine that immediate transport is necessary.
 - 2. Blood transfusions.
 - a. Transfusions shall be pre-existing in peripheral or central IV lines and started 15 minutes prior to transport. If the transfusion is started upon the arrival of transport, the transport will be delayed for 15 minutes to observe for any transfusion reactions.
 - 3. ATVs.
 - a. Paramedics shall not initiate ventilator support.

III. TRANSFERRING HOSPITAL PROCEDURES

- A. The paramedic shall receive written orders from the transferring physician prior to transport. These orders shall include a telephone number where the transferring physician can be reached during transport in addition to the following information:
 - 1. Magnesium sulfate, nitroglycerin (NTG), heparin, and/or amiodarone Infusions:
 - a. Type of solution.
 - b. Dosage and rate of infusion.
 - 2. Blood transfusions:
 - a. Blood type and unit identifying number.
 - b. Parameters for regulation of the transfusion rate.

3. ATVs:
 - a. Parameters for maintaining and adjusting ventilations during transport.
 - b. Orders for maintaining sedation with sedatives that are within the basic scope of practice for paramedics.
- B. The transferring hospital is responsible for mixing and labeling the infusion. If the existing infusion will not be sufficient for the transport duration, the hospital must provide additional clearly labeled per-mixed infusion(s).
- C. Transferring physicians must be aware and familiar with the basic paramedic scope of practice as well as the parameters for utilization of Paramedic IFT optional skills contained in Policies 6800, 6801, and 6802.